Fiscal Year:	FY 2026	**Task Last Updated:**	FY 10/31/2025
PI Name:	Clark, Torin K. Ph.D.		
Project Title:	A Non-Pharmacological Countermeasure Suite for Motion Sickness Induced by Post-Flight Water Landings		
Division Name:	Human Research		
Program/Discipline:			
Program/Discipline--Element/Subdiscipline:			
Joint Agency Name:		**TechPort:**	No
Human Research Program Elements:	(1) **HHC**:Human Health Countermeasures		
Human Research Program Risks:	(1) **Sensorimotor**:Risk of Altered Sensorimotor/Vestibular Function Impacting Critical Mission Tasks		
Space Biology Element:	None		
Space Biology Cross-Element Discipline:	None		
Space Biology Special Category:	None		
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Zip Code:	80303-7813	**Congressional District:**	2
Comments:	NOTE: PI moved to University of Colorado after NSBRI Postdoctoral Fellowship concluded in late 2015 (Ed., 9/1/17)		
Project Type:	Ground	**Solicitation / Funding Source:**	2019-2020 HERO 80JSC019N0001-HHCBPSR, OMNIBUS2: Human Health Countermeasures, Behavioral Performance, and Space Radiation-Appendix C; Omnibus2-Appendix D
Start Date:	01/01/2021	**End Date:**	09/30/2026
No. of Post Docs:	1	**No. of PhD Degrees:**	1
No. of PhD Candidates:	2	**No. of Master' Degrees:**	1
No. of Master's Candidates:	1	**No. of Bachelor's Degrees:**	3
No. of Bachelor's Candidates:	5	**Monitoring Center:**	NASA JSC
Contact Monitor:	Brocato, Becky	**Contact Phone:**	
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Flight Program:			
Flight Assignment:	NOTE: End date changed to 09/30/2026 per V. Lehman/JSC (Ed., 10/8/25). NOTE: End date changed to 09/30/2025 per C. Ribeiro/JSC (Ed., 6/24/24).		
Key Personnel Changes/Previous PI:	November 2025 report: None.		
COI Name (Institution):	DiZio, Paul Ph.D. (Brandeis University) Lawson, Benton Ph.D. (Self) Oman, Charles Ph.D. (Massachusetts Institute of Technology)		
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Task Description:	To mitigate astronaut motion sickness during capsule water landings, we aim to assess the benefit of providing Earth-fixed, external visual references, and enabling active postural control to increase head and torso stability, in a series of ground-based laboratory experiments. Re-exposure to Earth gravity, combined with the passive motion of the capsule in the sea is expected to cause varying degrees of motion sickness in most astronauts. In our laboratory experiments, we will use sustained hyper-gravity centrifugation and a visual reorientation paradigm to mimic adaptive responses to gravity-transitions experienced by astronauts. Immediately following, we will use our motion simulators to expose subjects to passive motions relevant for those expected for a capsule at sea. With the standard Motion Sickness Questionnaire, we will first quantify the prevalence, severity, and time course of resulting motion sickness. Next, we will systematically evaluate approaches which have been reported, mostly anecdotally, to benefit terrestrial seasickness, theoretically by helping anticipate the incoming sensory information and reducing the resulting sensory conflict. This includes 1) providing external visual reference cues within the capsule and 2) requiring the subject to try to keep their head and/or torso upright during the passive simulated sea-motion. We hypothesize external visual references will help subjects anticipate inertial motion cues (e.g., vestibular) that are otherwise unpredictable in a closed capsule. Given the emerging relationship between posture and motion sickness, we hypothesize subjects with their head and torso unrestrained and required to maintain alignment with upright during the passive motion stimulation will again help reduce sensory conflict and thus mitigate motion sickness. While these approaches are anecdotally-promising and grounded in sensory conflict theory, they have not been systematically assessed for the scenario of post-flight water landings. Through our experimental evaluations, we will develop a better scientific understanding of the mechanisms of motion sickness induced by post-flight water landings. Our planned countermeasure approaches are readily implementable within the capsule (e.g., providing external visual cues with projection displays or virtual reality) and should have no side effects. In fact, we hypothesize our non-pharmaceutical approaches can lead to reduced dosages of anti-motion sickness medications (e.g., promethazine), which do have undesirable side effects. If successful, these approaches will have substantial significance in reducing astronaut motion sickness post-water landings, which can otherwise impair mission performance and egress.
Rationale for HRP Directed Research:	
Research Impact/Earth Benefits:	This project focuses on developing countermeasures to mitigate motion sickness experienced by astronauts during water landings post-flight. While our focus is on the unique combination of astronauts experiencing a gravity transition (microgravity to 1 Earth g) along with the passive motion of the capsule produced by ocean waves, our approaches are likely to translate well to terrestrial motion sickness scenarios (e.g., seasickness, carsickness). While terrestrial motion sickness does not include the gravity transition experienced by astronauts, the passive, ocean wave motion is similar to that which often causes some forms of terrestrial motion sickness. Thus, we anticipate that our most promising countermeasures may be effective in helping mitigate some forms of terrestrial motion sickness. To help assess this, we will perform testing with subject cohorts that are exposed to 1) the gravity transition analog, 2) the wave-like motion analog, and 3) the combination of both, which will help us disambiguate the relative contributions of each, but also evaluate the countermeasures during just wave-like motion without the gravity transition (which may be more applicable for terrestrial motion sickness). Since motion sickness is commonly experienced in cars, boats, airplanes, and other paradigms like virtual reality, countermeasures to mitigate motion sickness could have substantial terrestrial benefits.
Task Progress:	In summary, we aim to use ground-based analogs to simulate the gravity transition of returning to Earth after long-duration spaceflight and then, in a laboratory setting, aim to mimic the wave-like motions expected during water landings. At the University of Colorado-Boulder, all subjects experienced the “sickness induced by centrifugation” (SIC) paradigm on our short-radius centrifuge capable of providing 2Gs for approximately 1 hour. SIC aims at mimicking a gravity transition relevant for spaceflight. Immediately following the SIC paradigm, subjects experience up to 1 hour of wave-like motion on our Tilt-Translation Sled. These profiles are representative of buoy data near potential water landing sites in terms of frequency content, amplitudes, and coherence of tilt motion versus lateral translation. The Brandeis arm of this project employs a virtual rendition of the visual reorientation levitation illusion introduced by Howard to partially simulate orbital microgravity, and a six-degree-of-freedom Stewart platform to simulate a water landing sea state (heave ±42 cm centered on 0.17 Hz and roll ±10° centered on 0.4 Hz). Sessions begin with the microgravity analog: one hour in which supine subjects make paced roll head movements in a fully articulated virtual room pitched back 90°. This is then followed by an immediate transition to the sea state analog: reorientation to the upright with head restraint and the onset of platform oscillation. Subjects are then tested in the sea state motion simulation with different potential countermeasures vs. control conditions. At the University of Colorado-Boulder, we have employed 1) a visual countermeasure group, who was provided a rich visual scene in virtual reality that moved congruently with their self-motion during the simulated “wave-like” motion, and 2) a control group, who also wore a head mounted display, but which displayed a stationary (in the field of view) fixation point, and thus no cues regarding self-motion. 3) an additional control group, who also had the stationary fixation point, but only experienced the wave-like motion without the preceding SIC exposure. (This second control group aims to isolate the impact of the SIC exposure vs. just the wave-like motion in inducing motion sickness), 4) an “active posture” countermeasure group, in which subjects have their head and torso unrestrained and are instructed to make postural motions to attempt to keep their head aligned with the direction of down during the wave-like motion, and 5) an “anticipatory cues” group, which were presented a novel display which not only provides a rich visual scene that moves congruently with self-motion (as in the visual countermeasure group previously), but also provides a figurine that moves in an anticipatory manner, providing cues of the upcoming motion 1 second into the future. Specifically, the display includes a semi-transparent notional figurine of a person, which tilts and translates laterally along a track to convey upcoming motion. In addition, an arrow points relative to the person in the direction (and relative magnitude) of the net gravito-inertial (i.e., the combination of linear acceleration and gravity), which is how the person may “feel” the net force acting upon them. We hypothesized that this would help subjects build a proper expectation of upcoming sensory information, reducing sensory conflict, and helping to mitigate motion sickness. At Brandeis, for the wave-like motion, subjects viewed either 1) a completely dark field, 2) a head-fixed single fixation point in a dark field, or 3) a virtual spatially stabilized horizon line. To determine whether active postural stabilization is an incremental countermeasure to an Earth-fixed visual reference, we are currently assessing motion sickness severity in the same ground-based 0g-to-sea state sequence, with the same horizon-fixed visual reference, when subjects are instructed to hold their head and torso upright versus when head and torso are restrained in the upright orientation. At the University of Colorado, we have completed testing of the visual countermeasure group (n=15), first control group (n=15), second control group (n=13), active posture group (n=15), and anticipatory cues group (n=12) at the University of Colorado. As a primary metric, we considered the proportion of subjects able to “survive” without experiencing “moderate” nausea on two successive reports during the wave-like motion (which, when achieved, served as our stopping criteria). While only 33% of subjects were able to “survive” throughout in the control condition, we found 90% were able to complete testing in the anticipatory cues group ($p=0.004$). On the other hand, the active postural group had only 53% of participants complete the full 60 minutes of wave-like motion, which was not statistically different than the control group. Stars represent when a subject’s testing was halted due to technical reasons and not due to the severity of their motion sickness. Previously at Brandeis, we found that the availability of a horizon-fixed visual reference mitigated motion sickness severity, relative to viewing a head-fixed visual target or being in complete darkness during exposure to partially simulated wave-like motion following sensitization by active head movements in a 0g analog environment. No additional progress was made at Brandeis within the last reporting year period. To date, we have found evidence of the anticipatory cues countermeasure (as well as the external visual references countermeasure) being effective at mitigating the proportion of individuals who experience “moderate” nausea during the hour of wave-like motion simulation. This approach, using virtual reality, is likely feasible to provide external visual references in terms of mass, power, and volume. We note that the anticipatory cues group was shown a depiction of how they would move one second into the future, overlaid with transparency over a rich visual scene. This raises the question as to what the relative contribution of the anticipatory cue visualization was, as compared to the rich visual scene of their current motion. To date, the anticipatory cue visualization has not been evaluated in isolation, which will be the focus of our upcoming experiment.
Bibliography Type:	Description: (Last Updated: 11/28/2025)
Abstracts for Journals and Proceedings	Katz J, Lonner TL, Gopinath AR, Clark TK. "Impact of sickness induced by centrifugation of motion sickness in a water landing analog." 2025 NASA Human Research Program Investigator’s Workshop, Galveston, Texas, January 28-31, 2025. Abstracts. 2025 NASA Human Research Program Investigator’s Workshop, Galveston, Texas, January 28-31, 2025. , Jan-2025

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Abstracts for Journals and Proceedings	Lonner TL, Gopinath AR, Clark TK. "Visual anticipatory cues as a method of reducing the incidence and severity of motion sickness." 2025 Vestibular Oriented Research Meeting, Boulder, Colorado, May 19-22, 2025. Abstracts. 2025 Vestibular Oriented Research Meeting, Boulder, Colorado, May 19-22, 2025. , May-2025
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